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PSYCHIATRIC ASSESSMENT MASTERY



Featuring

DSM-5-TR Decision Trees

Includes Full MSE Checklists

Step-by-Step Interview Scripts

Clinical Reasoning & SOAP Guides

Download the Digital Toolkit & Templates at:

www.psychmastery.com

This workbook bridges the gap between textbook theory and real-world clinical practice. Built specifically for **PMHNP** students, **RNs**, and new practitioners, it provides practical scripts, templates, and decision-making frameworks to support safe, structured, and efficient psychiatric assessment.

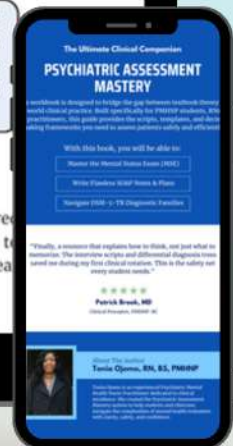
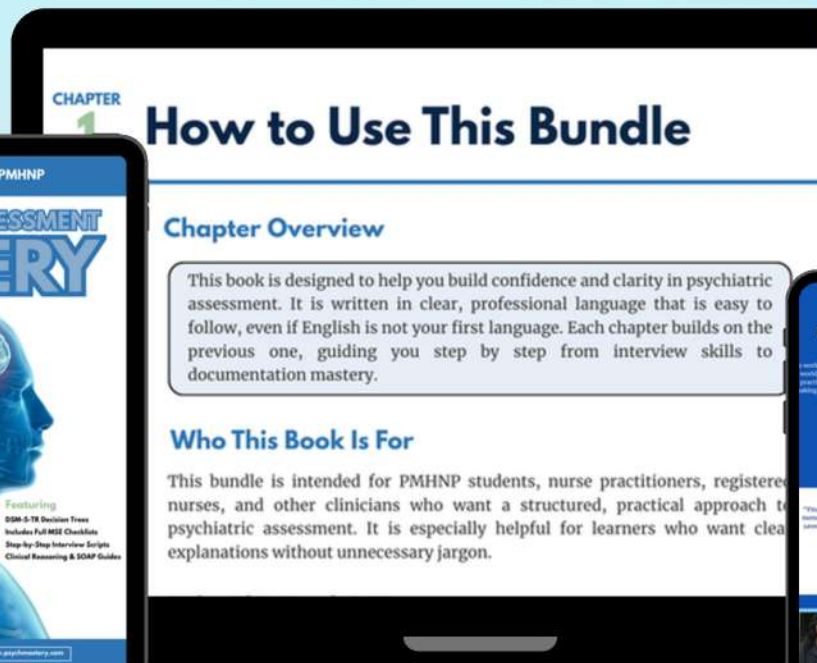
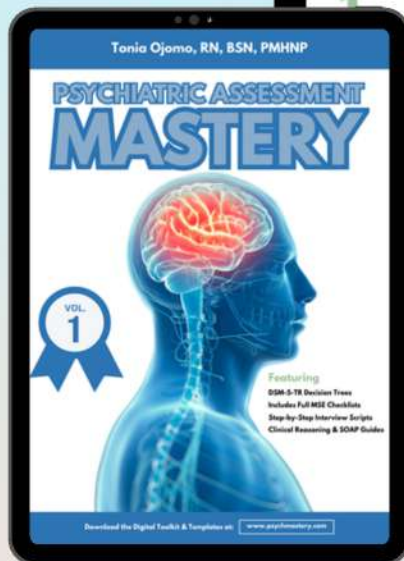


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SCAN THE QR CODE TO GET A DIGITAL BUNDLE



What This Is / What This Is Not



WHAT THIS IS

- ▶ **Volume 1 of Psychiatric Assessment Mastery:** A focused guide on Psychosis & Mood Disorders.
 - ▶ **A Clinical Workflow System:** Designed to improve psychiatric interviewing, MSE documentation, diagnostic reasoning, and **SOAP** writing.
 - ▶ **For Active Learners:** Built for PMHNP students, NPs, RNs, and clinicians who want a repeatable, structured framework for practice.
 - ▶ **Real-World Support:** A tool meant to be used during clinical practicum, exam preparation, and daily documentation.
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WHAT THIS IS NOT

- ▶ **Not a Substitute for Supervision:** Always follow your institutional policies, clinical protocols, and attending physician/supervisor guidance.
 - ▶ **Not a DSM-5-TR Replacement:** This book teaches pattern recognition and clinical reasoning. You must still verify specific criteria in the full DSM-5-TR text.
 - ▶ **Not a Theory Book:** This is not a text you read once and put away. It is a workbook designed for active use and repetition.
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How to Think Like a Psychiatric Clinician

A Simple Guide for Clear Clinical Thinking

Thinking like a psychiatric clinician does not mean using big medical words or memorizing the DSM. It means learning how to:

- ✓ Listen carefully
- ✓ Notice patterns
- ✓ Ask the right questions
- ✓ Stay calm and curious
- ✓ Connect symptoms to possible explanations

Start With Curiosity, Not Diagnosis

One of the most common mistakes new clinicians make is trying to name a diagnosis too quickly. Your job is to understand the patient's story first. The diagnosis comes later.

Instead of asking:

What diagnosis is this?

Ask:

What is this person experiencing right now?

Use This Simple 4-Step Thinking Process

Step 1: Listen to the Story

- ✓ Listen carefully
- ✓ Notice patterns
- ✓ Ask the right questions
- ✓ Stay calm and curious

Step 2: Observe What You See

- ✓ Appearance and behavior
- ✓ Eye contact, speech speed and tone
- ✓ Mood and emotional expression
- ✓ Movement (restless, slowed, calm)

Step 3: Identify Key Patterns

- ✓ Do the symptoms cluster together?
- ✓ Is there a clear timeline?
- ✓ Is there an immediate safety risk?
- ✓ How is the patient's functioning being affected?

Step 4: Ask, "What Could Explain This?"

- ✓ Could this be depression?
- ✓ Medication side effects?
- ✓ Anxiety or Trauma-related?
- ✓ Medical causes or Substance use?



THINK IN POSSIBILITIES, NOT ABSOLUTES

Psychiatric assessment is not about being 100% certain at the first visit. It is about:

- ✓ Identifying the most likely possibilities
- ✓ Ruling out dangerous conditions
- ✓ Creating a safe plan
- ✓ Reassessing over time

SAFETY ALWAYS COMES FIRST

If safety is a concern, address it before anything else. No matter the diagnosis, always assess for:

- ✓ Suicidal thoughts
- ✓ Homicidal thoughts
- ✓ Psychosis
- ✓ Severe mood instability
- ✓ Substance withdrawal



Use Plain Language in Your Own Mind

This patient feels sad most days, has low energy, poor sleep, and has stopped enjoying life.



The above is better thinking than:

Patient meets criteria for major depressive disorder.

Clinical Thinking Improves with Practice

Confidence grows with repetition. No one becomes a strong psychiatric clinician overnight. You will improve by:

- ✓ Practicing assessments
- ✓ Reviewing cases
- ✓ Using templates and checklists
- ✓ Reflecting on what worked and what did not

Final Reminder



Clear thinking

Careful listening

Organized notes



Perfect English.

Sound like a textbook

All the answers

That is what makes a good psychiatric clinician.



Quick Start: The 3-Step Assessment Workflow

1

Start with S (Subjective)

- ▶ **Chief Complaint:** Why are they here today?
- ▶ **HPI:** Onset, duration, severity, and triggers.
- ▶ **Safety:** Ask directly about SI/HI.
- ▶ **History:** Review medical, psychiatric, and social context.

2

Move to O + MSE (Objective)

- ▶ **Appearance & Behavior:** Grooming, eye contact, agitation.
- ▶ **Speech & Affect:** Rate, tone, volume, and emotional range.
- ▶ **Cognition:** Thought process, insight, and judgment.
- ▶ **Safety Signs:** Objective markers of risk or intoxication.

3

Write A + P (Assessment + Plan)

- ▶ **Diagnosis:** Primary diagnosis + key differentials/rule-outs.
- ▶ **Risk Level:** Low, Moderate, or High (with rationale).
- ▶ **Plan:** Medications, therapy, education, and follow-up timing.

How to Use This Workbook in Clinicals



BEFORE THE PATIENT ENCOUNTER

- ☒ **Review Question Banks:** Check the "Symptom Quick Prompts" (Chapter 13) for the patient's presenting problem (e.g., Depression vs. Anxiety).
 - ☒ **Refresh the MSE:** Skim the "Mental Status Exam" checklist (Chapter 4) to remember what to observe.
 - ☒ **Prep Your Template:** Open the HPI or SOAP template so you are ready to take structured notes.
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AFTER THE PATIENT ENCOUNTER

- ☒ **Draft the Note:** Fill in the SOAP template while the details are fresh in your mind.
 - ☒ **Check Your Wording:** Use the "Phrase Bank" (Chapter 19) to convert vague notes into professional medical language.
 - ☒ **Verify Safety:** Double-check that you documented SI/HI assessment and Risk Level explicitly.
 - ☒ **Self-Reflect:** Identify one specific area (e.g., "asking about trauma") to improve in the next encounter.
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